

PATIENT NAME: John Doe (Mock Patient)

DOB: 05/14/1967 (Age: 58)

SEX: Male

DATE OF SERVICE: 05/03/2026

PROVIDER: [Provider Name], MD

ENCOUNTER TYPE: Office Visit - Follow-up

CHIEF COMPLAINT: Routine follow-up for Type 2 Diabetes Mellitus and Essential Hypertension.

HISTORY OF PRESENT ILLNESS (HPI):

58-year-old male presents for a routine 3-month follow-up for management of Type 2 DM and primary hypertension. Patient reports feeling generally well with no acute complaints. He denies polyuria, polydipsia, chest pain, shortness of breath, or dizziness. Reports adherence to current medication regimen (Metformin and Lisinopril) with no adverse side effects. Home blood pressure readings over the last two weeks have been ranging in the 130s/80s. He is currently walking 30 minutes, 3 days a week, and attempting a low-sodium diet.

REVIEW OF SYSTEMS (ROS):

- **Constitutional:** Negative for fatigue, fever, or unexplained weight loss.
- **Eyes:** Negative for blurry vision or vision changes.
- **Cardiovascular:** Negative for palpitations, chest pain, or orthopnea.
- **Neurological:** Negative for numbness, tingling, or burning pain in the bilateral lower extremities.
- **Genitourinary:** Negative for dysuria or frequency.

OBJECTIVE (PHYSICAL EXAM):

- **Vitals:** BP: 138/88 mmHg | HR: 74 bpm | RR: 16 breaths/min | Temp: 98.4°F | SpO2: 98% on room air | BMI: 28.2
- **General:** Well-developed, well-nourished male in no acute distress.
- **HEENT:** Normocephalic, atraumatic. PERRLA. EOMI.
- **Cardiovascular:** Regular rate and rhythm. No murmurs, rubs, or gallops.
- **Pulmonary:** Clear to auscultation bilaterally. No wheezes, rales, or rhonchi.
- **Extremities:** No cyanosis, clubbing, or edema.
- **Neurological / Diabetic Foot Exam:** Bilateral lower extremities reveal intact sensation to 10g monofilament testing. No skin breakdown, ulcers, or calluses noted. Dorsalis pedis pulses 2+ bilaterally.

LABORATORIES / DIAGNOSTICS:

- **Point of Care HbA1c:** 7.2% (Target < 8.0%)
- **Recent BMP:** Serum creatinine 0.9 mg/dL, eGFR >60 mL/min/1.73m2. Potassium 4.2 mEq/L.
- **Urine Microalbumin/Creatinine Ratio:** Pending.

ASSESSMENT:

1. **Type 2 Diabetes Mellitus without complications (E11.9):** Adequately controlled. HbA1c is 7.2%, meeting standard targets. Neuropathy screening is negative today.
2. **Essential (Primary) Hypertension (I10):** Fairly controlled. Clinic BP today is 138/88, meeting the <140/90 threshold. Renal function remains stable on ACE inhibitor.

PLAN:

- **Medications:** * Continue Metformin 1000 mg PO BID.
 - Continue Lisinopril 20 mg PO daily.
- **Preventative Care & Screenings:**
 - *Retinopathy Screening:* Referral placed for annual diabetic dilated eye exam.
 - *Nephropathy Screening:* Will review spot urine microalbumin/creatinine ratio once results return.
- **Lifestyle:** Encouraged continued regular aerobic exercise (target 150 mins/week) and adherence to a low-sodium, diabetic-friendly diet.
- **Follow-up:** Return to clinic in 3 months with a repeat HbA1c prior to the visit. Patient instructed to call the office if he experiences hypoglycemia symptoms, dizziness, or other acute concerns.